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Can I Still Comfort a *Distressed Child?*

Safe, appropriate touch in early childhood

by Developmental Educator and
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Why I Wrote This Guide

Over the past year, I have found myself having more and more conversations with early childhood educators who are trying incredibly hard to keep children safe, follow changing expectations and make thoughtful decisions in moments that often happen very quickly, yet underneath those conversations I keep hearing the same uncertainty coming through.

“Am I still allowed to comfort a child when they are really distressed?”

“Can I pick them up when they reach for me?”

“Is it okay for a child to sit on my lap when they are having a difficult morning?”

“What do I do with the child who seems to settle when they receive firm physical reassurance, but I am suddenly worried that I might be doing the wrong thing?”

What has stayed with me is that these questions are not being asked by educators who do not care about child safety. In most cases, it is the complete opposite. They are being asked by educators who care so deeply about doing the right thing that they have started second guessing interactions that once felt natural, responsive and connected.

I can understand why this is happening, particularly when child safety conversations have become stronger, policies are being reviewed and educators are hearing more about boundaries, appropriate conduct and the importance of protecting children from harm. These conversations are necessary, and I would never want us to move away from them, but I also think there is another conversation that needs to sit alongside them.

Young children have not suddenly stopped needing warm, predictable and responsive adults simply because our understanding of child safety has become stronger.

A three year old who is crying at drop off does not understand legislation or the nervousness an educator may now be feeling about whether they should offer physical comfort. They are communicating something through their body, their facial expression, their movement and sometimes through reaching towards the adult they trust, and our role is to become better at noticing what that child is communicating rather than becoming so frightened of making a mistake that we stop responding altogether.

That does not mean every child needs a hug, every distressed child should be picked up, or that physical touch is always the right response.



Why I Wrote This Guide

Sometimes the safest and most supportive thing we can do is sit nearby, soften our voice, reduce the amount of language we are using, offer a familiar object, change the environment or simply give the child enough space to work out what they need from us.

The important part is that we are making those decisions thoughtfully, rather than from fear. As you work through this guide, I would love you to think about the children in your own setting and notice the different ways they ask adults for connection. Some will reach their arms towards you, some will move closer without saying anything, some will ask directly for a cuddle, while others may become louder, busier or more physical when what they are really struggling to communicate is that they no longer feel settled or safe.

There is no single response that will be right for every child, and that is exactly why I wanted to create this resource.

My hope is that it gives educators and leaders a calmer place to have these conversations together, so that instead of each person privately wondering whether they are getting it right, teams can begin building a shared understanding of what safe, responsive and respectful support can look like in everyday practice.

This guide is not legal advice, and every service will still need to follow its own policies, Approved Provider guidance and the requirements of its regulatory authority, but I hope it helps us move away from fear and towards thoughtful practice, where child safety and genuine human connection are allowed to strengthen one another rather than being treated as competing priorities.

Thinking of you,

Robyn Papworth





When Good Educators Start Second Guessing Themselves

I have actually put off writing this guide for quite a while, because this year I have found myself doing something I did not expect to be doing after so many years in this field. I have been questioning parts of the trauma-informed practice, behaviour support training and co-regulation knowledge that I have spent years learning, teaching and applying alongside children, families and educators.

Not because I suddenly believe that the need for connection has disappeared, or that children no longer benefit from calm, responsive adults, but because the conversations around child safety have become stronger and, at times, harder to translate into the everyday moments where educators have to make decisions quickly.

I have caught myself revisiting practices that I have understood for years and asking, “Is this still okay?”, “How would this be interpreted now?”, or “Could an educator feel nervous doing this, even when the child is clearly seeking comfort?”

That has been confronting, because I know I am not the only one doing it.

When experienced educators, leaders and professionals start second guessing themselves, it tells me there is a gap between the intent of child safety reform and the practical understanding people need in order to carry that intent into real relationships with children.

The difficulty is that children do not arrive in neat, easy-to-interpret situations.

A child might run towards an educator for a hug at drop off. Another might climb onto a familiar adult's lap after becoming overwhelmed. One child might need more space and very little physical contact, while another may move their whole body closer when they are tired, frightened or unsure. A child who has fallen may reach both arms towards the educator they trust, and another child may become louder or more physical when what they are really struggling to communicate is that they need help to feel settled again.

These moments require more than a rule that says “touch” or “do not touch”.



When Good Educators Start Second Guessing Themselves

They require us to notice the child, understand the purpose of our response, consider what the child is communicating, and stay aware of the boundaries and responsibilities that protect both children and educators.

This is where I think many good educators are getting stuck.

They are not trying to ignore child safety. They are trying so hard to respect it that some have started becoming hesitant in moments where children are looking to them for reassurance, and when that hesitation is happening across a whole team, children can begin receiving very different responses depending on which adult happens to be beside them.

One educator might offer a cuddle when a child reaches towards them, while another might deliberately keep their distance because they are worried that comforting touch could be misunderstood. A third educator might want to help but feel so unsure about the boundaries that they wait for someone else to step in.

Nobody in that situation is necessarily doing something because they do not care.

They may simply be working from different interpretations of what safe practice now means. That is why I eventually decided I needed to stop waiting until I felt I had a perfectly neat answer and start this conversation instead.

I do not want this guide to tell educators that everything they have learned about trauma-informed care, relationships, attachment, behaviour support or co-regulation should be thrown away. I also do not want to dismiss the importance of stronger child safety expectations.

I want us to become better at holding both.

When Good Educators Start Second Guessing Themselves

We can protect children's bodily autonomy and still recognise when a child is actively seeking safe comfort.

We can have clear professional boundaries and still be warm.

We can question old practices without becoming frightened of every interaction.

We can become more thoughtful about touch without teaching educators that connection itself is dangerous.

And perhaps most importantly, we can talk about these moments openly as teams rather than leaving individual educators to quietly wonder whether they are getting it wrong.

As you move through this guide, I would encourage you to notice any situations where you have started hesitating more than you used to. You might even bring one of those moments to your next team conversation and ask, "What would each of us do here, and why?"

The goal is not for everyone to prove that they already know the correct answer.

The goal is to start building a shared language around safe, responsive practice, so that good educators can spend less energy second guessing themselves and more energy noticing what the child in front of them is actually communicating.



So What Has Actually Changed?



One of the reasons I think so many educators are feeling unsettled at the moment is that there have been several child safety changes introduced across a relatively short period of time, and when we hear words such as stronger requirements, inappropriate conduct, reporting obligations and child safe practice repeated around us, it can be very easy for all of that information to blend together into one much bigger feeling of, “I need to be extremely careful now.”

So I want to slow this part down and separate a few things.

From 1 September 2025, approved providers were required to notify their regulatory authority within 24 hours of incidents or allegations of physical or sexual abuse involving a child while being educated and cared for, rather than within the previous seven day timeframe.

Then, from 1 January 2026, the National Quality Standard strengthened its explicit focus on child safety within Quality Areas 2 and 7. Element 2.2.3 became Child Safety and Protection, with clearer wording around the responsibility of management, educators and staff to understand their roles in identifying and responding to children who may be at risk of abuse or neglect.

Quality Area 7 was also strengthened so that governance, leadership, policies and management systems more clearly reflect the responsibility of services to create and maintain a child safe culture.

Further National Law changes that commenced in February 2026 reinforced something that I think is especially important for this conversation: the safety, rights and best interests of children must be the paramount consideration in the decisions and actions of the adults involved in their education and care.

So What Has Actually Changed?



When I read that, I do not interpret it as an instruction for educators to stop being warm, responsive or physically available to children.

I interpret it as an invitation for us to become more thoughtful about why we are doing something, whose need it is meeting, what the child is communicating, and whether the response is appropriate for that particular child and situation.

That is quite different from simply deciding that all touch has suddenly become unsafe. For example, imagine a child arrives at kindergarten crying, moves directly towards a familiar educator and raises both arms to be picked up. Rather than beginning with, “Am I allowed to touch this child?”, I would like us to become more confident asking a fuller set of questions.

What is this child communicating to me?

Are they actively seeking closeness?

What support usually helps them during separation?

Can I respond in a way that respects their body, their needs and the professional boundaries of my role?

And if another educator walked into the room right now, would the reason for this interaction make sense?

Those questions do not remove child safety. They bring child safety into the interaction. This is the shift I hope we can start making together, because stronger safeguarding should help us become more intentional in our relationships with children, rather than so frightened of getting something wrong that we become less responsive to the children standing in front of us.

So What Has Actually Changed?



As you think about your own setting, it may be useful to ask your team one very simple question:

What have we started doing differently because we believe the child safety expectations have changed?

There may be some really important changes in that conversation that should absolutely remain.

There may also be a few things your team has quietly stopped doing through uncertainty rather than because anyone has actually told you that the practice is no longer appropriate.

Those are the areas worth exploring together.

The dates and NQS wording in this page are based on ACECQA's current guidance: the 24-hour reporting change commenced on 1 September 2025, and the strengthened Quality Areas 2 and 7 commenced on 1 January 2026.



Children Are Wired for Connection

Before we talk any further about when touch is appropriate or inappropriate, I think we need to remind ourselves of something that can easily get lost when we are concentrating so heavily on policies, boundaries and risk.

Human beings develop through relationships.

Young children learn about safety, trust, communication and their own emotions through thousands of small interactions with the adults around them, and those interactions are happening long before a child has the language to explain what they are feeling or what they need.

The science of early childhood development has consistently shown us that responsive relationships matter. When a young child cries, reaches, gestures, moves closer, looks towards an adult or tries to communicate something through their body, and a caring adult notices that signal and responds appropriately, that back and forth interaction becomes part of how the developing brain learns what relationships feel like.

Sometimes that response will be our voice, our facial expression or simply staying close. Sometimes it will be play, movement or helping the child change what is happening around them.

And sometimes, when the child is seeking it and the situation is appropriate, that response may include physical comfort such as holding a hand, sitting close, offering a cuddle or picking up a young child who has reached towards us.



Children Are Wired for Connection

This does not mean that children need constant physical touch, and it certainly does not mean that every child will experience touch in the same way. Some children actively seek closeness when they are distressed, while others regulate more effectively when adults give them additional space, and many children will want different things at different times.

That is why responsive care is not about deciding that touch is always good. It is about noticing the child well enough to recognise what kind of connection they are asking for.

I think this matters enormously in our current conversations about child safety, because if fear causes us to withdraw from children altogether, we risk losing sight of what decades of developmental science have also been telling us: children need safe, predictable and responsive relationships with adults. Our job is not to choose between connection and protection. It is to become increasingly skilled at providing both.

For your next team discussion, you might simply notice the different ways children in your setting seek connection throughout the day. Which children reach for a hand, which children move in for a cuddle, which children sit nearby without touching, and which children seem to feel safer when you give them more physical space?

There is a lot we can learn when we stop asking, “Do children need touch?” and begin asking, “How does this particular child let me know what safe connection feels like for them?”





Appropriate Touch Has Not Disappeared

This is probably the part of the guide I have been most eager to write, because I think some of the fear we are seeing has come from educators hearing stronger language around inappropriate physical contact and professional boundaries, and then understandably wondering whether the safest response is simply to reduce physical contact with children altogether.

When I look more closely at the guidance though, that is not what I believe we are being asked to do.

Current guidance talks about touch being child led, purposeful, connected to the child's health, safety and wellbeing, and, where possible, happening in a way that is visible and understandable within the normal environment of the service.

For me, that gives us something much more useful to work with than deciding that touch is either completely safe or completely unsafe.

Imagine a child falls outside, becomes upset and runs towards a familiar educator with both arms raised. The educator crouches down, offers a brief cuddle, checks that the child is okay and stays nearby while they settle.

There is a clear reason for the interaction, the child has moved towards the educator, the adult is responding to what the child is communicating, and the contact is connected to the child's wellbeing in that moment.

That is very different from repeatedly encouraging a child to sit on an adult's lap when they have not sought that closeness, or continuing physical affection because the adult enjoys the interaction rather than because it is meeting a need being communicated by the child. This is why I think context matters so much.

The question is not simply, "Did touch happen?" The more useful question is whether the touch had a clear purpose, whether it made sense for that child and situation, and whether the child's response was being noticed and respected.



Appropriate Touch Has Not Disappeared

I also think it is important that we do not make the idea of child led touch too simplistic, because young children do not always have the words to clearly tell us what they need.

A toddler may not be able to say, “I am overwhelmed by Mum leaving this morning and I need you close,” but they may walk directly towards a familiar educator with both arms raised. A preschooler may not explain that the noise in the room has become too much, but they may quietly move closer and lean against the educator they trust. Another child may reach for an adult’s hand before entering a new space, while another may simply sit close enough that their shoulder touches the person beside them.

All of those children are communicating something.

Part of responsive practice is becoming better at noticing those quieter body cues, rather than waiting for children to find language they may not yet have. At the same time, child led does not mean that every child initiated interaction should automatically continue. The adult still holds responsibility for professional boundaries, context and safety, and we still need to notice whether the child remains comfortable once the contact begins.

For me, one of the most useful things to watch is what happens next. Does the child soften and stay close, remain relaxed and engaged, or do they stiffen, pull back, turn away or try to change position?

Sometimes the clearest information comes after the contact begins, because that is when the child’s body tells us whether the support we offered is actually helping.

This becomes especially important for children who use little or no spoken language, children who communicate differently, or children whose past experiences may shape how they respond to physical closeness.

We cannot assume that because a particular type of touch helped one child, it will be right for another child in the same situation. That is not a reason to become hesitant about every interaction. It is a reason to become more observant.



What Makes Touch Inappropriate?

When people hear the words inappropriate touch, it is easy for the mind to jump straight to the worst possible interpretation, but in everyday early childhood practice the more useful question is often not simply whether physical contact happened, but whether that contact was necessary, respectful, appropriate for the child and connected to a genuine need in that moment.

Current ACECQA guidance asks us to consider the circumstances around an interaction, including whether the contact reflects generally accepted education and care practice, whether it is appropriate for the child's age and developmental stage, whether it could cause harm, and whether the conduct is sexual, aggressive or violent. It also makes clear that physical contact that is invasive, or inappropriate for a child's age and developmental stage, may amount to inappropriate conduct even when a child appears to have agreed to it.

For me, this is where context matters so much, because two interactions can look similar from a distance while having very different purposes.

A child who has fallen and moves towards an educator for a brief cuddle is very different from an adult repeatedly encouraging physical closeness that the child has not sought. Helping a child move away from an immediate danger is very different from physically controlling them because they are not complying quickly enough, and sitting beside a distressed child who has moved close to you is very different from continuing physical contact once that child has started pulling away.

There may also be some familiar practices worth reflecting on as a team, not because educators have had harmful intentions, but because our understanding of children's bodily autonomy and professional boundaries continues to grow. This might include routinely inviting hugs as an expected greeting or goodbye rather than first noticing whether that child wants physical closeness, encouraging lap sitting when another position would work just as well, continuing tickling or physical play after the child's body language changes, or using touch mainly to gain compliance.

What I want educators to keep coming back to is whether the interaction still belongs to the child. If there is a clear reason for the contact, it makes sense for what is happening, the child's response is being noticed and respected, and the interaction would be understandable to another educator or family member observing it, we are thinking much more carefully than a simple rule of "touch" or "do not touch" ever allows.

That does not mean becoming suspicious of every warm interaction. It means making sure the contact remains purposeful, respectful and centred on the child.

When a Child Says No, But Safety is a Risk



There is another part of this conversation that I think we need to talk about openly, because respecting a child's body cues and listening when they say "no" does not mean that an educator can always step away when an immediate safety risk is developing.

There may be times when a child is highly distressed, frightened or overwhelmed and refuses an adult's direction, even though they are moving towards a road, attempting to climb over a fence, about to throw something towards another child, or behaving in a way that creates an immediate risk of harm to themselves or somebody nearby.

In those moments, we still take the child's "no" seriously, but we also have a responsibility to keep children safe.

Where there is enough time, I would always begin with the least intrusive response I can use, which may mean moving closer, reducing the amount of language I am using, positioning myself between the child and the danger, and giving the child an opportunity to move with me rather than immediately taking physical control. I might calmly say, "I can see that you don't want to come with me, but I need to keep you safe. Can you hold my hand and we'll move over here together?"

For a child whose nervous system is already highly activated, that small opportunity to participate can make an enormous difference, because rather than grabbing first, we are giving them a clear explanation, offering them some agency within the situation and showing them that our intention is to protect rather than punish or control.

Sometimes, though, there simply is not enough time for that approach to work before somebody is at risk of being hurt. In those moments, the question changes from "How can I encourage this child to move with me?" to "What is the minimum support I need to use right now to keep everyone safe?" That is where physical intervention may become necessary.





When Physical Intervention Becomes Necessary

There will be moments in early childhood settings where the risk develops so quickly that an educator does not have time to work through every calming strategy before stepping in. A child may be climbing over a fence, moving towards a road, lifting a heavy object towards another child, or becoming so physically unsafe that somebody is likely to be hurt if the adult waits.

In those moments, keeping everyone safe becomes the immediate priority. ACECQA is clear that physical restraint should only be used in an emergency situation, so if physical intervention becomes necessary, we should use only the physical support needed to interrupt the immediate danger and stop as soon as that risk has passed.

Wherever possible, we can still tell the child what is happening, even if our language needs to be very simple. “I’m going to help you move away from the fence because I need to keep you safe.” “I won’t let the block hit someone. I’m going to help you put it down.”

Those few words help the child understand that we are responding to safety rather than trying to punish or overpower them.

Once everybody is safe, what happens next matters too. We can reduce our language, soften our body position, give the child time to recover and reconnect without immediately expecting an apology or explanation while their nervous system is still highly activated.

Knowing the child well can also help us notice the smaller signs that appear before things become unsafe. If we know a child is likely to climb, bolt, throw or lash out when overwhelmed, we may be able to offer movement, more space, a quieter environment or a familiar adult earlier, before physical intervention becomes necessary.

If significant physical intervention has been required, educators should also follow their service procedures for recording what occurred, informing the appropriate leader and communicating with families where required.

That is the part I do not want us to lose. Keeping everyone safe matters, but so does what we learn from the moment afterwards.

A Simple Way to Think in the Moment



By this point in the guide, I hope you are beginning to feel that safe and appropriate touch is not about memorising a long list of rules, because in real early childhood settings there is rarely time to mentally work through a policy document while a child is crying at drop off, hiding under a table, reaching towards you for comfort, or becoming so overwhelmed that safety is suddenly at risk.

What we do need is a simple way to slow our thinking down just enough that our response becomes thoughtful rather than automatic, and the easiest way I have found to do that is to pause, notice and respond.

Pause

Pausing does not mean standing back and doing nothing, particularly when there is an immediate safety concern. It simply means giving ourselves a small moment to become aware of our own reaction before we move in, because when an educator is feeling rushed, worried, frustrated or frightened of getting something wrong, it is very easy to respond from our own nervous system rather than from what the child actually needs.

Sometimes that pause might only be one breath, softening our shoulders, lowering our voice, or noticing that our own body has started to tense before we respond.



A Simple Way to Think in the Moment



Notice

Then we notice what the child is communicating through their body, their movement, their voice and the way they are responding to us. Are they moving towards connection, pulling away from touch, looking for more space, reaching for a familiar adult, or becoming so activated that their safety or somebody else's safety is now part of the decision?

We can also notice what is happening around the child, because a response that makes sense during a quiet drop off may look very different when there are twenty children moving around the room, another child at risk of being hurt, or a child trying to leave a safe area.



Respond

Once we have noticed what is actually happening, we can choose the least intrusive response that still meets the child's needs and keeps everyone safe.

Sometimes that will mean sitting close without touching. Sometimes it will mean offering a hand, reducing our language, changing the environment or giving the child more space.

Sometimes a child will clearly move towards us for a cuddle or ask to be picked up. Sometimes an immediate safety risk will mean we need to step in physically, using the gentlest response that will keep everyone safe.



What matters is that we are becoming more deliberate about why we are responding in a particular way, that we continue noticing the child once the interaction begins, and that we are willing to adjust when their body tells us they need something different.

For me, that is where confidence starts to return, because we no longer need to choose between being frightened of touch and going back to doing things simply because that is how we have always done them. We can pause, notice the child in front of us, and make the most thoughtful decision we can in that moment. That is what safe, responsive practice looks like to me.



A Final Note

This guide has been written as a practical professional learning resource to help early childhood educators and leaders think more clearly about safe, appropriate and responsive physical contact with children.

It is not legal advice, and it is not intended to replace the policies, procedures or professional responsibilities of your service.

Every service should continue to follow the guidance of its Approved Provider, the National Quality Framework, ACECQA, its relevant regulatory authority, and any other legislation or requirements that apply within its state or territory.

Child safety guidance will continue to evolve, which is why I encourage leaders to use this resource as a starting point for thoughtful team conversations rather than as a stand-alone policy or set of rules.

The current National Quality Framework requires children's safety, rights and best interests to be the paramount consideration in decisions and actions within education and care services. Current ACECQA guidance also recognises that appropriate physical contact can form part of professional care when it is purposeful, respectful, responsive to the child, consistent with professional responsibilities and appropriate to the situation.

If something in this guide raises a question about your own service practice, I would encourage you to take that question back to your team, your Approved Provider and your current policies, because the strongest child safe cultures are not created by individual educators quietly trying to interpret everything alone. They are built when teams are able to talk openly, reflect together and develop a shared understanding of what safe, warm and responsive practice looks like for the children they care for.

My hope is that this guide gives you more confidence to have those conversations, while still leaving space to keep learning as guidance, practice and our understanding of children continue to develop.

Guidance and regulatory information have been checked as of August 2026.

Thinking of you,

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